MODULES 6 & 7 — 64-QUESTION NCLEX / HESI PRACTICE EXAM

Challenging, scenario-based questions | Multiple choice • Select all that apply • Bowtie • Linked case studies

Name: _____ Date: _____ Score: _____ / 64

Instructions: Choose ONE best answer for multiple-choice and case-study items. For SATA, select ALL correct choices. For bowtie items, select ONE condition, TWO actions, and TWO monitoring parameters. Answers and detailed rationales appear ONLY at the very end. Do not turn to the answer-key section until finished.

Module 6 | Reproduction

1. [MULTIPLE CHOICE] A 29-year-old at 7 weeks has right-sided pelvic pain, light spotting, shoulder pain, BP 88/52, HR 128. Which order should the nurse anticipate as the immediate priority?

A. Prepare for emergency stabilization and surgical evaluation

B. Schedule serial beta-hCG in 48 hours as an outpatient

C. Teach that spotting is expected with implantation

D. Administer oral analgesia and reassess in one hour

Answer: _____

2. [SELECT ALL THAT APPLY] A patient at 9 weeks asks which exposures are especially important to avoid during organogenesis. Select all appropriate teaching statements.

■ A. Avoid alcohol because no safe amount is established in pregnancy.

■ B. Review all prescription and over-the-counter medications with the prenatal clinician.

■ C. Avoid tobacco and secondhand smoke exposure.

■ D. A medication is safe whenever it is sold without a prescription.

■ E. Limit exposure to known radiation and toxic chemicals.

Answer: _____

3. [MULTIPLE CHOICE] A patient currently pregnant previously had one term singleton, one preterm twin birth with both children living, and a pregnancy loss at 10 weeks. Which GTPAL is correct?

A. G4 T1 P1 A1 L3

B. G5 T1 P2 A1 L3

C. G4 T1 P2 A1 L3

D. G4 T2 P1 A1 L3

Answer: _____

4. [MULTIPLE CHOICE] A 30-year-old has had irregular cycles and 14 months of unsuccessful attempts to conceive. Which initial nursing response best supports appropriate evaluation?

A. Recommend evaluation of both partners, including ovulatory and semen factors

B. Advise waiting another year because menstrual cycles are irregular

C. Assume the cause is anovulation and begin fertility medication

D. Recommend IVF before any diagnostic assessment

Answer: _____

5. [SELECT ALL THAT APPLY] A patient at 11 weeks reports heavy bleeding and cramping. Which findings warrant immediate escalation? Select all that apply.

■ A. BP 86/48 and HR 132

■ B. Saturating two large pads in an hour

■ C. Syncope when standing

■ D. Mild fatigue without hemodynamic changes

■ E. Fever 38.6 C with uterine tenderness

Answer: _____

6. [MULTIPLE CHOICE] A patient asks why a positive home pregnancy test is not classified as a positive/confirmatory sign of pregnancy. Which explanation is best?

A. hCG can be present without a viable intrauterine fetus; ultrasound visualization provides confirmation

B. Pregnancy tests become reliable only after 20 weeks

C. Only a pelvic examination can confirm pregnancy

D. Urine hCG cannot detect early pregnancy

Answer: _____

Module 6 | Sexuality

7. [MULTIPLE CHOICE] During an STI assessment, a patient quietly states their partner monitors their phone and becomes angry when condoms are mentioned. Which response is most appropriate?

A. Ask about immediate safety privately and offer confidential resources

B. Insist that the partner attend the remainder of the interview

C. Document that the patient refuses safer sex

D. Tell the patient to confront the partner tonight

Answer: _____

8. [SELECT ALL THAT APPLY] A sexually active patient has no symptoms but reports a new partner and inconsistent condom use. Which actions are appropriate? Select all that apply.

■ A. Assess partners, practices, protection, STI history and pregnancy intention

■ B. Offer guideline-based STI screening

■ C. Provide nonjudgmental condom counseling

■ D. Assure the patient testing is unnecessary without symptoms

■ E. Discuss confidentiality and partner notification if infection is identified

Answer: _____

9. [MULTIPLE CHOICE] A patient with a history of migraine with visual aura requests a combined estrogen-progestin contraceptive. What is the best nursing action?

A. Flag the estrogen-containing method as potentially contraindicated and discuss nonestrogen options with the prescriber

B. Recommend increasing estrogen dose to prevent headaches

C. Tell the patient aura is unrelated to contraception choice

D. Recommend a combined patch because it bypasses the stomach

Answer: _____

10. [MULTIPLE CHOICE] A 52-year-old reports dyspareunia and vaginal dryness after menopause, without bleeding. Which initial assessment is most appropriate?

A. Explore genitourinary symptoms, medications, sexual concerns and possible vaginal atrophy

B. Assume the symptoms indicate an STI

C. Advise that sexual activity must stop after menopause

D. Recommend antibiotic therapy without examination

Answer: _____

11. [SELECT ALL THAT APPLY] A nurse counsels a patient with newly diagnosed chlamydia. Which teaching points are appropriate? Select all that apply.

■ A. Complete the prescribed antibiotic regimen

■ B. Discuss partner evaluation and treatment

■ C. Avoid sexual activity until recommended treatment conditions are met

■ D. Stop treatment when discharge resolves

■ E. Discuss retesting and screening as recommended

Answer: _____

Module 6 | Gynecologic disorders

12. [MULTIPLE CHOICE] A 34-year-old reports progressively painful menses, deep dyspareunia, and difficulty conceiving; ultrasound is unremarkable. Which condition remains plausible?

A. Endometriosis

B. Uncomplicated bacterial vaginosis

C. Physiologic ovulatory pain alone

D. Cervical ectropion without symptoms

Answer: _____

13. [SELECT ALL THAT APPLY] A patient with heavy menstrual bleeding reports exertional dyspnea and dizziness. Which assessments are priorities? Select all that apply.

■ A. Orthostatic vital signs

■ B. CBC including hemoglobin and hematocrit

■ C. Pregnancy test when applicable

■ D. Bleeding amount and pad saturation

■ E. Only a fasting lipid panel

Answer: _____

14. [MULTIPLE CHOICE] A patient with suspected PID has fever, pelvic pain, and cervical motion tenderness. Which nursing action best prevents long-term reproductive harm?

A. Facilitate prompt prescribed antimicrobial treatment and partner management

B. Delay antibiotics until pain becomes severe

C. Recommend only a heating pad until cultures return

D. Explain that PID cannot affect fertility

Answer: _____

15. [MULTIPLE CHOICE] A 16-year-old has severe cramps beginning just before menses, normal pelvic examination, and no red flags. Which mechanism most likely explains primary dysmenorrhea?

A. Increased endometrial prostaglandins causing uterine contractions

B. Excess thyroid hormone causing endometrial atrophy

C. Persistent ovarian torsion

D. An active pelvic infection in all cases

Answer: _____

16. [SELECT ALL THAT APPLY] A patient with fibroids is scheduled for evaluation after months of heavy bleeding. Which findings support a clinically significant complication? Select all that apply.

■ A. Hemoglobin 8.4 g/dL

■ B. Fatigue and exertional tachycardia

■ C. Increasing pad use and clots

■ D. Normal oxygen saturation alone

■ E. Dizziness on standing

Answer: _____

17. [MULTIPLE CHOICE] A patient with abnormal uterine bleeding has BP 82/46, HR 138 and cool clammy skin. Which action takes priority over routine diagnostic scheduling?

A. Activate urgent hemorrhage response and establish circulatory support

B. Arrange a nonurgent pelvic ultrasound next week

C. Teach a menstrual diary before further assessment

D. Offer reassurance because bleeding is common

Answer: _____

Module 6 | Infertility and contraception

18. [MULTIPLE CHOICE] A patient with anovulatory infertility begins gonadotropin injections. They report rapid weight gain, marked abdominal distention, vomiting and reduced urine output. Which complication is most concerning?

A. Ovarian hyperstimulation syndrome

B. Expected ovulation discomfort

C. Normal luteal phase symptoms

D. Uncomplicated menstrual cramps

Answer: _____

19. [SELECT ALL THAT APPLY] A couple is preparing for a hysterosalpingogram (HSG). Which nursing points are appropriate? Select all that apply.

■ A. Assess possibility of pregnancy before the procedure

■ B. Explain that contrast outlines the uterine cavity and fallopian tubes

■ C. Assess for active pelvic infection

■ D. Tell the patient HSG directly measures sperm motility

■ E. Prepare the patient for possible cramping

Answer: _____

20. [MULTIPLE CHOICE] A patient asks which assisted reproduction technique directly injects a single sperm into an oocyte. Which answer is correct?

A. ICSI

B. IUI

C. HSG

D. Ovulation predictor testing

Answer: _____

21. [MULTIPLE CHOICE] A patient took levonorgestrel emergency contraception after unprotected sex and asks whether it also treats a possible STI exposure. Which response is best?

A. It does not treat or prevent STIs; offer exposure-based testing and prevention counseling

B. It prevents most STIs if taken within 72 hours

C. It replaces the need for condoms for the next month

D. It guarantees that pregnancy cannot occur

Answer: _____

22. [SELECT ALL THAT APPLY] During infertility counseling, which responses reflect appropriate patient-centered nursing care? Select all that apply.

■ A. Invite discussion of emotional distress and treatment goals

■ B. Explain potential for multiple gestation with some treatments

■ C. Include both partners in evaluation when applicable

■ D. Promise successful pregnancy after the first IVF cycle

■ E. Discuss costs, alternatives and the option to stop treatment

Answer: _____

23. [MULTIPLE CHOICE] A patient using combined hormonal contraception develops sudden unilateral leg swelling and chest discomfort. What is the priority?

A. Urgent evaluation for possible venous thromboembolism

B. Reassure that leg swelling is an expected adjustment

C. Suggest taking the pill with food

D. Schedule routine follow-up in six months

Answer: _____

Module 7 | Prenatal care

24. [SELECT ALL THAT APPLY] At the first prenatal visit, which findings warrant focused follow-up rather than routine documentation only? Select all that apply.

■ A. Rh-negative blood type with unknown antibody status

■ B. History of intimate partner violence

■ C. Current use of an antiseizure medication

■ D. Rubella nonimmune status

■ E. A reliable last menstrual period alone

Answer: _____

25. [MULTIPLE CHOICE] A patient at 28 weeks becomes pale and nauseated while lying flat for an examination; BP falls from 118/72 to 88/50. Which action should the nurse take first?

A. Turn the patient to a lateral position

B. Raise both legs while maintaining supine position

C. Encourage prolonged breath holding

D. Immediately give oral glucose

Answer: _____

26. [MULTIPLE CHOICE] A patient’s last menstrual period began February 10. Using Naegele’s rule, what is the estimated due date?

A. November 17

B. October 17

C. November 3

D. December 10

Answer: _____

27. [SELECT ALL THAT APPLY] A patient at 32 weeks calls about symptoms. Which reports require prompt evaluation? Select all that apply.

■ A. Sudden gush of clear vaginal fluid

■ B. Marked decrease in usual fetal movement

■ C. Bright-red vaginal bleeding

■ D. Mild ankle edema after standing that resolves with rest

■ E. Regular painful contractions before 37 weeks

Answer: _____

28. [MULTIPLE CHOICE] At 25 weeks, the fundal height measures 19 cm on two careful measurements. What is the best nursing response?

A. Report the discrepancy and anticipate assessment of dating, fetal growth and fluid

B. Document as expected without follow-up

C. Diagnose fetal growth restriction from fundal height alone

D. Increase the patient’s daily salt intake

Answer: _____

29. [MULTIPLE CHOICE] A pregnant patient is rubella nonimmune at 12 weeks. Which plan is appropriate?

A. Avoid live rubella vaccine during pregnancy and arrange postpartum immunization

B. Administer MMR today to protect the fetus

C. Give rubella immune globulin at every visit

D. Defer all future vaccination indefinitely

Answer: _____

Module 7 | Maternal nutrition

30. [SELECT ALL THAT APPLY] A pregnant patient follows a strict vegan diet and has a history of bariatric surgery. Which nutritional risks merit assessment? Select all that apply.

- ■ A. Vitamin B12 deficiency
- ■ B. Iron deficiency
- ■ C. Protein inadequacy
- ■ D. Folate and other micronutrient deficiency
- ■ E. Guaranteed excess vitamin D stores

Answer: _____

31. [MULTIPLE CHOICE] At 18 weeks, a patient reports eating clay daily and feeling exhausted. Which laboratory assessment is most directly relevant?

A. CBC and iron studies

B. Serum troponin

C. Amylase and lipase only

D. Fasting lipid profile

Answer: _____

32. [MULTIPLE CHOICE] A patient with prepregnancy BMI 31 has gained 25 lb at 27 weeks in a singleton pregnancy. Which nursing response is best?

A. Review the weight trend, nutrition and activity, and collaborate on individualized counseling

B. Tell the patient to fast for 48 hours

C. State that no weight gain is recommended with obesity

D. Assume the gain proves gestational diabetes

Answer: _____

33. [SELECT ALL THAT APPLY] Which foods or exposures should a pregnant patient be taught to avoid or limit? Select all that apply.

- ■ A. Unpasteurized soft cheese
- ■ B. Undercooked meat
- ■ C. Alcohol
- ■ D. Low-mercury fully cooked salmon
- ■ E. High-mercury fish

Answer: _____

34. [MULTIPLE CHOICE] A patient at 10 weeks asks why folate is emphasized before conception and early in pregnancy. Which explanation is best?

A. Neural tube closure occurs early, often before pregnancy is recognized

B. Folate prevents every congenital anomaly

C. Folate primarily prevents third-trimester hemorrhage

D. Folate substitutes for adequate dietary protein

Answer: _____

Module 7 | Antepartum testing

35. [MULTIPLE CHOICE] At 34 weeks, an NST shows baseline 145 bpm, moderate variability, and only one 15-by-15 acceleration in 20 minutes. What is the best interpretation?

A. The test is not yet reactive; continue or obtain further evaluation per protocol

B. The test is reactive because one acceleration is sufficient

C. A positive CST has occurred

D. Fetal death is confirmed

Answer: _____

36. [MULTIPLE CHOICE] A CST at 37 weeks shows late decelerations after 4 of 5 contractions. How should the nurse interpret the result?

A. Positive, concerning for uteroplacental insufficiency

B. Negative, reassuring

C. Reactive, reassuring

D. Normal because decelerations occur after contractions

Answer: _____

37. [MULTIPLE CHOICE] A 30-week patient with fetal growth restriction has abnormal umbilical artery Doppler flow. What is the clinical concern?

A. Increased placental vascular resistance and impaired fetal perfusion

B. Confirmation of a fetal chromosomal disorder

C. Definitive diagnosis of maternal diabetes

D. Evidence that amniotic fluid is excessive

Answer: _____

38. [SELECT ALL THAT APPLY] A patient at 16 weeks is scheduled for amniocentesis. Which nursing actions are appropriate? Select all that apply.

■ A. Verify informed consent and explain postprocedure warning signs

■ B. Assess Rh status and need for Rh immune globulin if indicated

■ C. Observe for fluid leakage, bleeding, contractions or fever afterward

■ D. Tell the patient amniocentesis is only a screening test

■ E. Use ultrasound guidance as ordered

Answer: _____

39. [MULTIPLE CHOICE] A patient has a positive cell-free DNA screen for trisomy 21. What is the most accurate counseling statement?

A. The result estimates increased risk and diagnostic testing can be discussed

B. The screen definitively establishes the diagnosis

C. A normal anatomy ultrasound rules out the result

D. A positive result requires immediate delivery

Answer: _____

40. [MULTIPLE CHOICE] A patient at 35 weeks has low amniotic fluid and recurrent variable decelerations. Which mechanism best links these findings?

A. Umbilical cord compression due to reduced cushioning

B. Fetal anemia from excessive amniotic fluid

C. Maternal hyperthyroidism causing late decelerations

D. Increased fetal swallowing from maternal dehydration

Answer: _____

Module 7 | Preexisting disorders

41. [MULTIPLE CHOICE] A 22-week pregnant patient with type 1 diabetes has glucose 190 mg/dL, positive ketones, pH 7.24, nausea and tachypnea. What is the priority?

A. Treat as possible DKA despite glucose below typical nonpregnant thresholds

B. Withhold urgent care until glucose exceeds 300 mg/dL

C. Encourage exercise to lower ketones

D. Give only oral carbohydrates and discharge

Answer: _____

42. [SELECT ALL THAT APPLY] A pregnant patient with pregestational diabetes is reviewing the care plan. Which points are appropriate? Select all that apply.

■ A. Frequent glucose assessment and medication adjustment may be needed

■ B. Insulin requirements can change as pregnancy progresses

■ C. Maternal glucose control influences neonatal hypoglycemia risk

■ D. Fetal growth surveillance may be indicated

■ E. Stop all insulin during illness to avoid hypoglycemia

Answer: _____

43. [MULTIPLE CHOICE] A patient with asthma at 29 weeks has wheezing, SpO2 89%, and difficulty speaking full sentences. Which priority is correct?

A. Support maternal oxygenation and obtain urgent respiratory treatment

B. Avoid prescribed bronchodilators because all are unsafe in pregnancy

C. Wait for a fetal ultrasound before treating maternal hypoxemia

D. Give oral fluids and schedule a follow-up

Answer: _____

44. [MULTIPLE CHOICE] A patient with chronic hypertension develops new proteinuria, rising creatinine and severe headache at 31 weeks. Which interpretation is most concerning?

A. Superimposed preeclampsia

B. Uncomplicated chronic hypertension

C. Normal physiologic renal adaptation

D. Isolated white-coat hypertension

Answer: _____

45. [SELECT ALL THAT APPLY] A pregnant patient with a seizure disorder asks about medications. Which nursing approaches are appropriate? Select all that apply.

■ A. Review antiseizure therapy with the obstetric and neurology teams

■ B. Discourage abrupt self-discontinuation of antiseizure medications

■ C. Discuss folate supplementation and individualized monitoring

■ D. Explain uncontrolled seizures can endanger maternal and fetal health

■ E. Advise stopping all antiseizure medications immediately after a positive pregnancy test

Answer: _____

46. [MULTIPLE CHOICE] A patient with cardiac disease develops new dyspnea at rest, crackles, tachycardia and SpO2 90%. What should the nurse do first?

A. Escalate for possible pulmonary edema and support oxygenation

B. Reassure that all dyspnea is physiologic in pregnancy

C. Encourage supine bed rest and delay evaluation

D. Administer additional IV fluids without assessment

Answer: _____

Module 7 | Gestational complications

47. [MULTIPLE CHOICE] At 33 weeks, BP is 164/112 twice 15 minutes apart, with visual spots and epigastric pain. Which prescription should the nurse expect?

A. Urgent antihypertensive treatment and seizure prophylaxis

B. Oral iron and routine follow-up in four weeks

C. Diuretic therapy as the only treatment

D. No action until 24-hour urine protein returns

Answer: _____

48. [SELECT ALL THAT APPLY] A patient receiving magnesium sulfate has RR 10/min, absent DTRs, urine output 15 mL/hr and increasing somnolence. Which actions are indicated? Select all that apply.

■ A. Stop the magnesium infusion

■ B. Call for urgent assistance and assess airway/respirations

■ C. Prepare calcium gluconate as prescribed

■ D. Increase the magnesium rate to prevent seizures

■ E. Evaluate renal function and urine output

Answer: _____

49. [MULTIPLE CHOICE] A patient at 35 weeks has severe RUQ pain, platelets 62,000/mm3, AST 220 U/L and evidence of hemolysis. Which syndrome best explains the pattern?

A. HELLP syndrome

B. Uncomplicated gestational hypertension

C. Iron deficiency without pregnancy complication

D. Physiologic thrombocytopenia alone

Answer: _____

50. [MULTIPLE CHOICE] A patient at 32 weeks has sudden constant abdominal pain, dark vaginal bleeding, rigid uterus and recurrent late decelerations. Which condition is most likely?

A. Placental abruption

B. Placenta previa

C. Uncomplicated Braxton Hicks contractions

D. Physiologic cervical change

Answer: _____

51. [SELECT ALL THAT APPLY] A patient at 30 weeks has painless bright-red bleeding and ultrasound suggests placenta previa. Which actions are appropriate? Select all that apply.

■ A. Avoid digital vaginal examination

■ B. Monitor maternal vital signs and blood loss

■ C. Assess fetal status

■ D. Prepare IV access and blood availability as indicated

■ E. Perform routine digital cervical examination to measure dilation

Answer: _____

52. [MULTIPLE CHOICE] A patient with suspected abruption is bleeding from IV sites, fibrinogen is very low, platelets are falling and PT is prolonged. Which complication is most likely?

A. Disseminated intravascular coagulation

B. Isolated iron deficiency

C. Normal third-trimester hypercoagulability

D. Uncomplicated urinary tract infection

Answer: _____

Clinical judgment | Bowtie

53. [BOWTIE] At 34 weeks a patient has BP 170/114, severe headache, visual disturbances, 3+ DTRs and clonus. Select the most likely condition, TWO priority actions, and TWO monitoring parameters.

MOST LIKELY CONDITION — select 1

■ A. Preeclampsia with severe features

■ B. Placenta previa

■ C. Uncomplicated gestational hypertension

PRIORITY ACTIONS — select 2

■ A. Initiate seizure precautions and obtain urgent obstetric help

■ B. Prepare ordered magnesium sulfate and antihypertensive therapy

■ C. Encourage ambulation to relieve headache

■ D. Defer evaluation until a 24-hour urine is complete

MONITORING PARAMETERS — select 2

■ A. Respiratory rate and DTRs

■ B. Urine output and BP trends

■ C. Daily calorie intake only

■ D. Pap smear status

Condition: _____ Actions: _____ , _____ Monitor: _____ , _____

54. [BOWTIE] At 9 weeks a patient reports sharp unilateral pelvic pain, shoulder-tip pain and syncope. HR 134, BP 78/42, abdomen rigid. Select the likely condition, TWO priority actions, and TWO parameters to monitor.

MOST LIKELY CONDITION — select 1

- ■ A. Ruptured ectopic pregnancy
- ■ B. Hyperemesis gravidarum
- ■ C. Uncomplicated miscarriage

PRIORITY ACTIONS — select 2

- ■ A. Activate emergency response and prepare rapid IV resuscitation
- ■ B. Prepare for urgent surgical evaluation
- ■ C. Schedule routine ultrasound next week
- ■ D. Encourage the patient to walk to reduce pain

MONITORING PARAMETERS — select 2

- ■ A. BP, pulse and mental status
- ■ B. Hemoglobin and ongoing blood loss
- ■ C. Routine fundal height
- ■ D. Maternal weight gain over one month

Condition: _____ Actions: _____ , _____ Monitor: _____ , _____

55. [BOWTIE] At 28 weeks a patient with type 1 diabetes has vomiting, abdominal pain, RR 30, pH 7.21, serum ketones positive and glucose 205 mg/dL. Select the condition, TWO actions and TWO monitoring priorities.

MOST LIKELY CONDITION — select 1

- ■ A. Pregnancy-associated DKA
- ■ B. Isolated morning sickness
- ■ C. Gestational hypertension

PRIORITY ACTIONS — select 2

- ■ A. Activate urgent DKA treatment pathway with IV fluids and insulin per orders
- ■ B. Assess electrolytes and obtain maternal-fetal monitoring
- ■ C. Encourage exercise to lower glucose
- ■ D. Delay treatment until glucose exceeds 300

MONITORING PARAMETERS — select 2

- ■ A. Potassium and acid-base status
- ■ B. Glucose, urine output and fetal status
- ■ C. Only daily weight after discharge
- ■ D. Cervical dilation as sole indicator

Condition: _____ Actions: _____ , _____ Monitor: _____ , _____

56. [BOWTIE] At 36 weeks a patient has painless bright-red bleeding, soft nontender uterus, and a prior ultrasound showing a low-lying placenta. Select the condition, TWO actions and TWO monitoring priorities.

MOST LIKELY CONDITION — select 1

- ■ A. Placenta previa
- ■ B. Placental abruption
- ■ C. Eclampsia

PRIORITY ACTIONS — select 2

- ■ A. Avoid digital vaginal examination
- ■ B. Establish IV access and prepare hemorrhage resources
- ■ C. Perform digital exam to assess dilation
- ■ D. Encourage walking to advance labor

MONITORING PARAMETERS — select 2

- ■ A. Maternal BP, pulse and bleeding
- ■ B. Fetal heart rate pattern
- ■ C. Patellar reflexes only
- ■ D. Urine ketones only

Condition: _____ Actions: _____ , _____ Monitor: _____ , _____

57. [BOWTIE] A patient receiving gonadotropins for infertility has abdominal distention, weight gain of 4 kg in 3 days, tachycardia, oliguria and shortness of breath. Select the complication, TWO actions and TWO monitoring priorities.

MOST LIKELY CONDITION — select 1

- ■ A. Ovarian hyperstimulation syndrome
- ■ B. Uncomplicated ovulation
- ■ C. Primary dysmenorrhea

PRIORITY ACTIONS — select 2

- ■ A. Urgently notify fertility team and assess cardiopulmonary status
- ■ B. Prepare ordered fluid and electrolyte evaluation
- ■ C. Reassure that rapid weight gain is always normal
- ■ D. Encourage vigorous exercise

MONITORING PARAMETERS — select 2

- ■ A. Urine output and daily weight
- ■ B. Respiratory status and abdominal girth
- ■ C. Tanner stage only
- ■ D. Routine Pap results

Condition: _____ Actions: _____ , _____ Monitor: _____ , _____

58. [BOWTIE] At 33 weeks a patient has severe abdominal pain, boardlike uterus, vaginal bleeding, fetal bradycardia and falling fibrinogen. Select the condition, TWO actions and TWO monitoring priorities.

MOST LIKELY CONDITION — select 1

- ■ A. Placental abruption with possible consumptive coagulopathy
- ■ B. Placenta previa without hemorrhage
- ■ C. Normal prodromal labor

PRIORITY ACTIONS — select 2

- ■ A. Activate obstetric hemorrhage/emergency response
- ■ B. Prepare for rapid maternal resuscitation and urgent birth as directed
- ■ C. Perform repeated digital exams before calling
- ■ D. Delay care pending a routine outpatient ultrasound

MONITORING PARAMETERS — select 2

- ■ A. Maternal hemodynamics and coagulation studies
- ■ B. Continuous fetal heart rate
- ■ C. Postpartum contraception choice only
- ■ D. Fundal height next month

Condition: _____ Actions: _____ , _____ Monitor: _____ , _____

Next Generation NCLEX | Case Study A

59. [CASE STUDY] CASE A: A 27-year-old G1P0 at 31 weeks arrives with headache unrelieved by acetaminophen, flashing lights, RUQ pain, BP 166/108 and 168/110 15 minutes apart, platelets 88,000/mm3, AST 146 U/L, creatinine 1.3 mg/dL (baseline 0.6). FHR baseline 145 with moderate variability. Which interpretation best integrates these data?

A. Preeclampsia with severe features and concern for HELLP spectrum

B. Gestational hypertension without severe features

C. Normal physiologic changes of pregnancy

D. Uncomplicated migraine without obstetric concern

Answer: _____

60. [CASE STUDY] CASE A: A 27-year-old G1P0 at 31 weeks arrives with headache unrelieved by acetaminophen, flashing lights, RUQ pain, BP 166/108 and 168/110 15 minutes apart, platelets 88,000/mm3, AST 146 U/L, creatinine 1.3 mg/dL (baseline 0.6). FHR baseline 145 with moderate variability. The nurse receives orders for magnesium sulfate and IV labetalol. Which assessment finding requires the most immediate action during the infusion?

A. RR 9/min with absent patellar reflexes

B. BP 146/92 after antihypertensive treatment

C. FHR baseline 145 with moderate variability

D. Urine output 45 mL over the past hour

Answer: _____

61. [CASE STUDY] CASE A: A 27-year-old G1P0 at 31 weeks arrives with headache unrelieved by acetaminophen, flashing lights, RUQ pain, BP 166/108 and 168/110 15 minutes apart, platelets 88,000/mm3, AST 146 U/L, creatinine 1.3 mg/dL (baseline 0.6). FHR baseline 145 with moderate variability. After stabilization, the patient asks why the team is discussing delivery before the due date. Which response is best?

A. Severe disease can progress to maternal organ injury and fetal compromise; timing balances both risks

B. A reassuring fetal heart rate excludes all maternal complications

C. The presence of edema alone requires immediate delivery

D. Delivery is discussed only because the patient requested it

Answer: _____

Next Generation NCLEX | Case Study B

62. [CASE STUDY] CASE B: A 32-year-old G3P1 at 35 weeks has insulin-treated diabetes and reports reduced fetal movement since yesterday. NST over 40 minutes is nonreactive. Ultrasound BPP is 4/10 with low amniotic fluid. BP 126/78, afebrile. Which action should the nurse prioritize?

A. Promptly notify the obstetric team and prepare for further evaluation and possible delivery planning

B. Reassure that a normal BP makes fetal compromise unlikely

C. Schedule routine repeat testing in one month

D. Tell the patient to stop insulin until movement returns

Answer: _____

63. [CASE STUDY] CASE B: A 32-year-old G3P1 at 35 weeks has insulin-treated diabetes and reports reduced fetal movement since yesterday. NST over 40 minutes is nonreactive. Ultrasound BPP is 4/10 with low amniotic fluid. BP 126/78, afebrile. Which explanation best connects low fluid to a possible intrapartum fetal heart rate pattern?

A. Reduced cushioning can increase cord compression and variable decelerations

B. Low fluid invariably produces maternal bradycardia

C. Low fluid is diagnostic of fetal trisomy

D. Low fluid prevents any uterine contractions

Answer: _____

64. [CASE STUDY] CASE B: A 32-year-old G3P1 at 35 weeks has insulin-treated diabetes and reports reduced fetal movement since yesterday. NST over 40 minutes is nonreactive. Ultrasound BPP is 4/10 with low amniotic fluid. BP 126/78, afebrile. After birth, the newborn is jittery and has poor feeding. Which assessment should the nurse anticipate first?

A. Check neonatal blood glucose promptly

B. Wait until the next scheduled routine feeding

C. Assess maternal platelet count as the sole priority

D. Measure maternal fundal height before neonatal evaluation

Answer: _____

ANSWER KEY & CLINICAL RATIONALES

STOP: This section is intentionally placed after all 64 questions. For SATA, every listed letter is required. For bowtie items, use the three-part selection key.

1. A

Shock and referred shoulder pain suggest ruptured ectopic pregnancy with intra-abdominal hemorrhage.

2. A,C,B,E

Embryonic organ formation is highly sensitive to teratogens; OTC status does not establish safety.

3. A

Each multifetal delivery counts as one pregnancy and one birth for parity, but two living children.

4. A

Infertility evaluation addresses both partners and considers endocrine, tubal, uterine and sperm factors.

5. A,B,C,E

Hemodynamic compromise, significant bleeding, and infection signs are urgent; mild isolated fatigue is less immediately threatening.

6. A

A positive hCG is a probable sign, not definitive confirmation of a viable intrauterine pregnancy.

7. A

Private, trauma-informed safety assessment takes priority when coercion or intimate partner violence is possible.

8. A,B,C,E

Asymptomatic infections are common; the 5 Ps guide risk assessment and individualized screening.

9. A

Migraine with aura is a major contraindication to combined hormonal contraception due to ischemic stroke risk.

10. A

Estrogen-related genitourinary changes may cause dryness and pain, but assessment must remain individualized.

11. A,B,C,E

Treatment completion, partner management, abstinence until appropriate, and retesting reduce reinfection.

12. A

Endometriosis may cause cyclic pelvic pain and infertility even with normal routine ultrasound.

13. A,B,C,D

Hemodynamic assessment, anemia evaluation, pregnancy exclusion and quantifying loss guide urgent care.

14. A

Delayed PID treatment raises risk of tubal damage, infertility and ectopic pregnancy.

15. A

Prostaglandin-mediated uterine contractions drive primary dysmenorrhea; NSAIDs target this pathway.

16. A,B,C,E

Fibroid-related heavy bleeding can produce symptomatic iron-deficiency anemia and volume depletion.

17. A

Shock requires immediate stabilization before nonurgent etiologic testing.

18. A

OHSS may cause capillary leak, third spacing, oliguria and thromboembolic risk.

19. A,B,C,E

HSG evaluates uterine and tubal patency; pregnancy and infection are important precautions.

20. A

ICSI places one sperm inside an egg during assisted fertilization.

21. A

Emergency contraception addresses pregnancy risk, not infection; efficacy is not absolute.

22. A,B,C,E

Fertility care includes realistic counseling, psychosocial support and shared decisions without guaranteeing outcomes.

23. A

Estrogen-containing contraception can increase VTE risk; leg symptoms with chest discomfort require emergency evaluation.

24. A,B,C,D

Rh/antibody status, safety, teratogenicity and immunity affect management; a reliable LMP supports dating.

25. A

Lateral positioning relieves inferior vena cava compression from the gravid uterus.

26. A

Subtract three months and add seven days: February 10 becomes November 17.

27. A,B,C,E

Membrane rupture, decreased fetal movement, bleeding and possible preterm labor require assessment.

28. A

A persistent fundal-height discrepancy needs evaluation; measurement alone cannot establish the diagnosis.

29. A

MMR is live attenuated and contraindicated during pregnancy; postpartum vaccination is appropriate.

30. A,B,C,D

Dietary restrictions and altered absorption increase risk for multiple deficiencies; excess vitamin D is not guaranteed.

31. A

Pica and fatigue may accompany iron-deficiency anemia.

32. A

Recommended total gain for obesity is generally 11–20 lb; counseling should be individualized and nonjudgmental.

33. A,B,C,E

Foodborne pathogens, alcohol and mercury exposure are risks; properly cooked low-mercury fish provides useful nutrients.

34. A

Folate is essential for early neural tube development, before many patients know they are pregnant.

35. A

At 32 weeks or later, reactivity generally requires two qualifying accelerations; a nonreactive initial tracing needs further assessment.

36. A

Late decelerations with at least half of contractions constitute a positive CST.

37. A

Umbilical artery Doppler evaluates placental resistance and fetal blood flow in growth restriction.

38. A,B,C,E

Amniocentesis is diagnostic; ultrasound guidance, Rh precautions and postprocedure monitoring support safety.

39. A

Cell-free DNA is a screening rather than a diagnostic test.

40. A

Oligohydramnios increases cord-compression risk, producing variable decelerations.

41. A

DKA in pregnancy can occur at lower glucose levels and threatens both maternal and fetal well-being.

42. A,B,C,D

Pregnancy changes insulin needs; sick-day management and surveillance are essential rather than stopping insulin automatically.

43. A

Maternal hypoxemia is an immediate threat to both mother and fetus; treat the exacerbation promptly.

44. A

New organ dysfunction and proteinuria after 20 weeks in chronic hypertension raise concern for superimposed preeclampsia.

45. A,B,C,D

Abrupt withdrawal may precipitate seizures; coordinated medication and fetal-risk assessment is essential.

46. A

New hypoxemia and crackles suggest cardiopulmonary decompensation.

47. A

Persistent severe-range BP and severe features demand urgent treatment without waiting for protein quantification.

48. A,B,C,E

Respiratory depression, absent reflexes and oliguria indicate magnesium accumulation/toxicity.

49. A

Hemolysis, elevated liver enzymes and low platelets define HELLP.

50. A

Pain, uterine rigidity, bleeding and fetal compromise are classic for abruption.

51. A,B,C,D

Digital examination may trigger catastrophic hemorrhage in placenta previa.

52. A

Consumptive coagulopathy after abruption causes low fibrinogen, thrombocytopenia, prolonged clotting and bleeding.

53. Condition A; Actions A,B; Monitor A,B

Neurologic symptoms with severe hypertension signal severe preeclampsia; treatment and toxicity/hemodynamic monitoring are essential.

54. Condition A; Actions A,B; Monitor A,B

Hemorrhagic shock in early pregnancy requires immediate stabilization and source control.

55. Condition A; Actions A,B; Monitor A,B

DKA in pregnancy can occur with modest hyperglycemia and requires fluids, insulin, electrolytes and fetal assessment.

56. Condition A; Actions A,B; Monitor A,B

Previa causes painless bleeding; vaginal examination can worsen hemorrhage.

57. Condition A; Actions A,B; Monitor A,B

OHSS can cause capillary leak, ascites, intravascular depletion and respiratory compromise.

58. Condition A; Actions A,B; Monitor A,B

Abruption may cause fetal hypoxia, maternal shock and DIC; simultaneous maternal and fetal stabilization is critical.

59. A

Severe-range systolic BP, neurologic and hepatic symptoms, thrombocytopenia and renal injury indicate severe disease.

60. A

Respiratory depression and loss of reflexes suggest magnesium toxicity; stop infusion and respond urgently.

61. A

Delivery is the definitive treatment for preeclampsia, with timing guided by severity, gestation and stabilization.

62. A

Reduced movement, nonreactive NST, low fluid and BPP 4/10 warrant urgent fetal evaluation.

63. A

Oligohydramnios reduces protection against intermittent cord compression.

64. A

Infants of diabetic mothers are at risk of neonatal hypoglycemia due to persistent hyperinsulinemia after birth.

Educational practice examination based on the uploaded Modules 6 and 7 chapter topics. Clinical actions must follow facility policy, current standards and provider orders.